



CLINICIAN SUMMARY

What this profile may indicate.

Self-report results show mild-range depression indicators, mild-range anxiety indicators, and mild-range anger indicators, with mild-range global distress. The pattern is concentrated in dysphoria, unsustained effort, negative cognition, and fatigue rather than evenly distributed across domains. A non-zero critical-screen response must be reviewed directly with the person before this summary is approved or used in care planning. These findings are hypotheses for a licensed clinician to test against interview data, history, context, and observed functioning; they do not establish a diagnosis.

RECORD	FF-TEST-EQ-001
COMPLETED	2026-08-13T18:04:45.000Z
COVERAGE	105 of 105
SOURCE	e-QPASS proposed RFI synthetic scored event

01 Core dimensions

Scored e-QPASS output stays authoritative.

DEPRESSION 35 MILD	ANXIETY 20 MILD	ANGER 27 MILD	GLOBAL INDEX 82 MILD
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SAFETY ROUTING	Direct critical-screen review acknowledged	1 HIGHLIGHTED SOURCE RESPONSE
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02 Pattern checks

Comparison prompts, not conclusions.

MIXED SIGNALS No automated mismatch Range labels are internally aligned in this record; interview context may still reveal complexity not captured by scores.	No specific score mismatch identified	RED FLAGS Direct critical-screen review required A qualified clinician must verify the authoritative source response and complete direct safety assessment before approval or care-planning use. 1 highlighted source response; raw response wording is intentionally not reproduced.
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03 Clinical themes

Interview hypotheses, not diagnostic conclusions.

01 ANXIETY Worry-led anxiety pattern MODERATE CONFIDENCE Mild anxiety indicators are distributed across apprehension, interpersonal concerns, and physiological arousal. Clarify persistence, triggers, avoidance, and functional impact. UNCERTAINTY The scored profile does not establish duration, cause, functional impact, or whether this signal persists outside the assessment context. Anxiety - 20 Apprehension - 8	02 ANGER Anger expression warrants contextual clarification MODERATE CONFIDENCE The anger-related signal is concentrated in specific expression constructs rather than sufficient on its own to establish a broader conclusion. Clarify triggers, frequency, consequences, and regulation strategies. UNCERTAINTY The scored profile does not establish duration, cause, functional impact, or whether this signal persists outside the assessment context. Angry mood - 6 Anger in - 5	03 CROSS-DOMAIN Profile concentration should be tested in context MODERATE CONFIDENCE The strongest subscale signals cluster in negative cognition, apprehension, and angry mood. Determine whether this pattern is persistent, situational, recent, or already changing before using it in care planning. UNCERTAINTY The scored profile does not establish duration, cause, functional impact, or whether the relationship among these signals persists outside the assessment context. Negative cognition - 8 Apprehension - 8 Angry mood - 6
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04 Questions for the next conversation

Use context and clinical judgment to test the pattern.

01 What does the person understand the non-zero critical-screen response to mean, and what direct safety assessment is indicated now?	02 What changed around the onset of reduced interest, energy, motivation, or self-evaluative strain?
03 Which situations most reliably activate worry, avoidance, interpersonal tension, or physiological arousal?	04 What tends to precede anger-related experiences, and how are they expressed, contained, or resolved?
05 What is the current effect, if any, on sleep, concentration, work, school, relationships, or daily decisions?	

Quality checks

Scored input coverage	100%
Diagnostic restraint	PASS
Theme evidence lineage	PASS
Critical-screen route	DIRECT REVIEW
Clinical specificity	NOT SCORED

Confidence + limitations

01 This is a single-timepoint self-report profile and may be affected by response style, context, comprehension, and willingness to disclose.
03 The scored record does not establish duration, onset, trajectory, functional impairment, protective factors, or collateral context.
05 Critical-screen status must be verified in the authoritative source record and addressed through direct clinical assessment.

NO OMITTED UNCERTAINTY

02 Range labels use the current synthetic calibration mapping and require confirmation against the authoritative e-QPASS scoring specification before live use.
04 PERL does not infer diagnosis, causation, treatment, medical necessity, eligibility, or level of care.

CLINICAL BOUNDARY

This structured decision-support summary is derived from self-report scores. It does not diagnose, prescribe, determine level of care, or replace interview data, history, functional assessment, direct safety assessment, or licensed clinical judgment.

Legal review pending · ff-clinical-disclaimer/draft-2026-08

APPROVED BY

LOCAL-GATE-REHEARSAL

Aug 13, 2026, 10:10 PM

PERL-CLINICIAN-REPORT/1.0 · PERL-CLINICAL-BRIEF/1.0

deterministic-calibration · cal-0.9.3

Source 90ac73502bb1 · Artifact 685544b130fd
Prompt deterministic-rules/cal-0.9.3 · Policy 23bc876878f9
Scoring synthetic-score-rules-2026-08 · Findings synthetic-findings-v1 · receipt 1658bc4da3a2